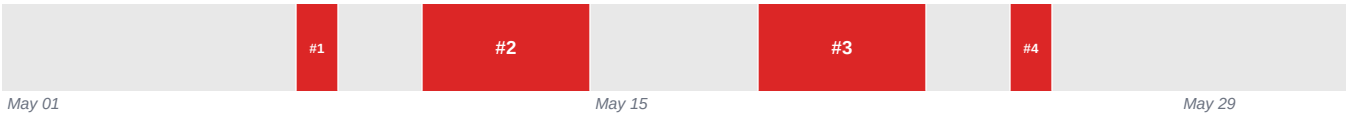


CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: Jones Period: May 1 to June 1, 2026 Report Date: June 2, 2026 Coverage: 704/745h (94%)
Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 32 days from May 01 to Jun 01 ■ HR ■ Breathing



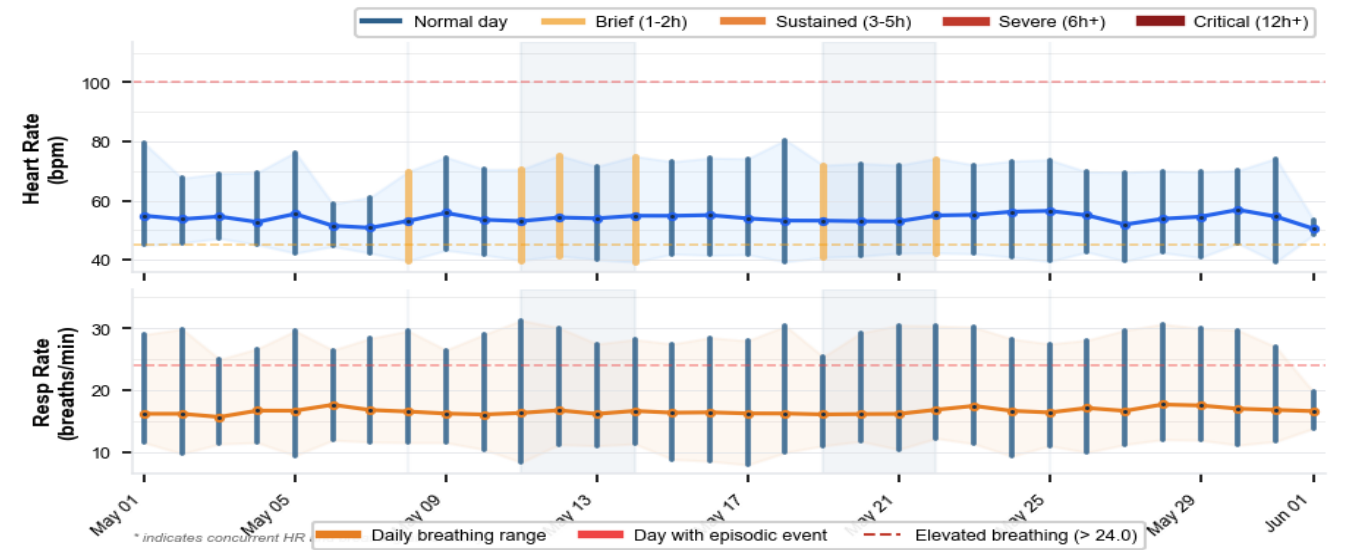
Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
9	12h	Low Heart Rate	<1	94%

Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Low Heart Rate	39 bpm	2h	2h	1	43 bpm	May 08
2	Low Heart Rate	39 bpm	4h	2h	<1	43 bpm	May 11 to May 14
3	Low Heart Rate	39 bpm	5h	2h	1	44 bpm	May 19 to May 22
4	Low Heart Rate	39 bpm	1h	1h	1	44 bpm	May 25

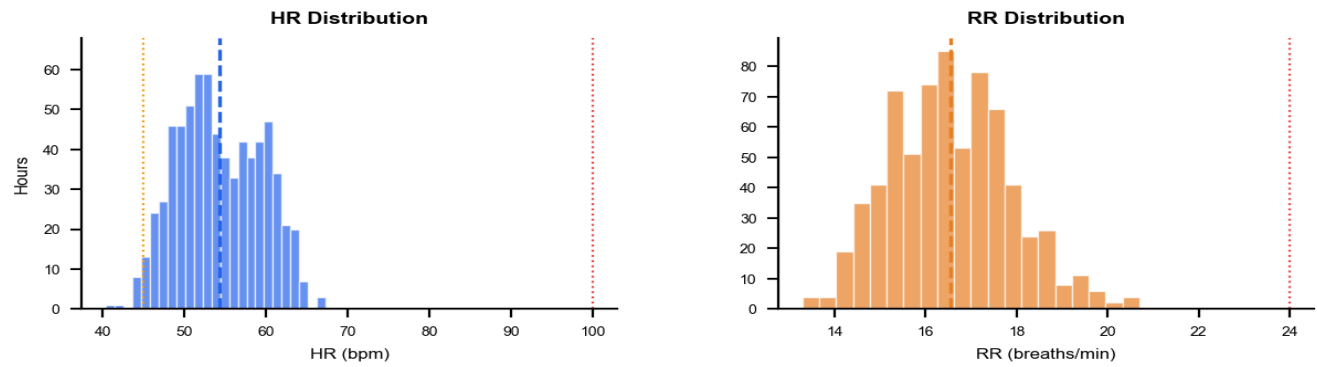
Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

- Clinical Pattern Observations:
- 1. **Clustered pattern:** Episodic events on 6 of 32 days (18%).
 - 2. **Nocturnal pattern:** 4 of 6 eligible episodes during evening or night hours.

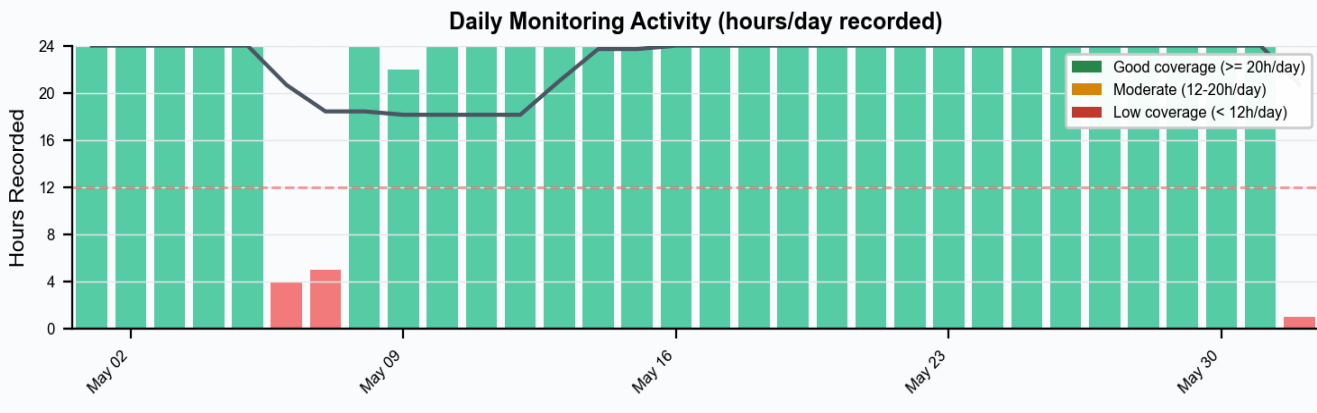


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

■ Very Low HR < 40 bpm ■ Low HR < 45 bpm ■ Elevated HR > 95 bpm ■ High HR > 100 bpm
■ Very High HR > 110 bpm ■ Elevated Breathing > 24 brpm ■ High Breathing > 30 brpm ■ Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.